

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Study in Participants With Active Systemic Lupus Erythematosus With Inadequate Response to Glucocorticoids and ≥ 2 Immunosuppressants **Short Title/Acronym:** Benchmark-SLE **Protocol Number:** CA061-1025 **NCT Number:** NCT07175285 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Current standard of care (SoC) treatment options **Phase of Development:** Observational Study **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of current Standard of Care (SoC) in achieving remission and improving clinical outcomes—specifically Definition of Remission in SLE (DORIS) at 6 months—in patients with active SLE (including Lupus Nephritis) refractory to glucocorticoids and at least 2 immunosuppressants. **Secondary Objectives:** To evaluate drug-free DORIS remission, complete renal response (CRR) for subjects with baseline Lupus Nephritis, maintenance of response (DORIS, LLDAS, SRI-4) at 6, 12, 24, 36, 48, and 60 months, SLICC/ACR damage index, incidence/severity of flares, changes in proteinuria/eGFR, and changes in Patient-Reported Outcomes (PRO) such as FACIT-Fatigue.

2.2 Background & Rationale Systemic lupus erythematosus (SLE) is a chronic, immune-mediated disease causing debilitating symptoms and organ damage. Patients with active SLE, including lupus nephritis (LN), frequently experience inadequate responses to glucocorticoids (GC) and immunosuppressants (IS), leading to refractory disease and poor outcomes. There are limited prospective data on the efficacy and safety of contemporaneous standard of care (SoC) in these populations. The Benchmark-SLE study aims to generate robust real-world evidence and up-to-date real-world benchmark rates of remission, response, and safety using comprehensive clinical trial outcome measures to contextualize data from other ongoing SLE clinical trials evaluating novel investigational agents.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target **\$N\$:** 223 adult patients
Number of Sites: Global, multicenter (approx. 34 locations)
Estimated Study Start: 08-Oct-2025 **Estimated Primary Completion:** 01-Mar-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults ≥ 16 years of age at the time of signing the ICF. 2. Meet the European League Against Rheumatism (EULAR) / American College of Rheumatology (ACR) 2019 classification criteria for systemic lupus erythematosus (SLE). 3. Have active disease at study entry defined as ≥ 1 BILAG A OR ≥ 1 BILAG B with a history of qualifying manifestations within 24 months, AND positive autoantibodies. 4. Have an inadequate response to therapeutic doses of glucocorticoids and ≥ 2 immunosuppressant therapies (e.g., cyclophosphamide, mycophenolic acid, belimumab, anifrolumab, rituximab, methotrexate, etc.) for at least 3 months each.

4.2 Exclusion Criteria 1. Pregnant women. 2. Participants enrolled in concurrent interventional clinical trials involving investigational therapies or any other clinical trial. 3. Participant is unwilling and unable to adhere to the study visit schedule and other protocol requirements.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Standard of Care (SoC) dosing as prescribed by the treating physician. **Route of Administration:** Oral, IV, or SC (Dependent on specific SoC modality). **Duration of Treatment:** Up to 60 months of observational follow-up.

5.2 Concomitant & Prohibited Therapy Permitted: Low-dose GCs, antimalarials, and/or stable immunosuppressants/biologics per standard clinical practice. **Prohibited:** Concurrent investigational therapies or participation in interventional clinical trials.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, EULAR/ACR criteria evaluation, Baseline SLEDAI/BILAG
Baseline	Day 0	Enrollment, SoC verification, Initial PRO collection
Interim	Months 6, 12, 24, 36, 48	Disease activity assessment (DORIS, LLDAS, SRI-4), optional renal biopsy at Month 6 for LN patients with persistent proteinuria, Safety/AE monitoring
End of Study	Month 60	Final clinical evaluation, SLICC/ACR damage index, Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of participants achieving Definition of Remission in Systemic Lupus Erythematosus (DORIS) at 6 months. **Measurement Method:** Clinical SLEDAI = 0, Physician Global Assessment (PhGA) < 0.5 (VAS 0-3), with permitted low-dose GCs, antimalarials, and/or stable IS/biologics.

7.2 Secondary & Exploratory Endpoints * Drug-free DORIS remission and complete renal response (CRR) for baseline LN subjects. * Maintenance/duration of response (LLDAS, SRI-4, DORIS) at 6, 12, 24, 36, 48, and 60 months. * Change in Patient-Reported Outcomes (PROs) including FACIT-Fatigue.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during the observational period; reporting of incidence of adverse events (AEs) including serious AEs up to 60 months. **Serious Adverse Events (SAEs):** Reported according to standard pharmacovigilance timelines. **Data Monitoring Committee (DMC):** Internal/Sponsor safety review handled by Bristol Myers Squibb.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All enrolled patients (intent-to-treat observational cohort, approx. N=223); data will be summarized descriptively. **Sample Size Justification:** Approximately 223 patients expected to provide robust real-world evidence and 95% Confidence Interval estimates for DORIS remission rates. **Handling of Missing Data:** Standard missing data handling algorithms for prospective observational cohorts (e.g., LOCF or multiple imputation for longitudinal outcome measurements).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Observational prospective data collection; periodic site monitoring for data completeness. **Audit Trail:** eCRF locking, source data verification for key composite endpoints (BILAG/SLEDAI) and data clarification forms.

11. Study Identifiers & Governance

Primary ID: CA061-1025 **Registry Number:** NCT07175285 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** Benchmark-SLE

Official Regulatory Title: A Study in Participants With Active Systemic Lupus Erythematosus With Inadequate Response to Glucocorticoids and ≥ 2 Immunosuppressants

12. Clinical Framework (SLE Specific)

Primary Condition: Systemic Lupus Erythematosus (SLE) and Lupus Nephritis (LN) **Diagnosis Threshold:** Active SLE (≥ 1 BILAG A or ≥ 1 BILAG B) refractory to GC and ≥ 2 IS **Medical Methodology:** Current Standard of Care (SoC) options **Optimization Target:** DORIS Remission at 6 months

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for SLE / Lupus Nephritis **Education Protocol (HCP):** Training on SLE composite endpoints (SLEDAI, BILAG, DORIS) **Education Protocol (Patient):** Patient-Reported Outcomes (PRO) completion (e.g., FACIT-Fatigue) **Quality Audit Mechanism:** eCRF source data verification and real-world data collection

14. Observation & Follow-Up Schedule

Total Duration: 60 months **Onsite Visit Cadence:** Months 6, 12, 24, 36, 48, 60 **Remote Monitoring Frequency:** Standard of Care follow-up **Checklist Review Interval:** Milestone intervals at 6, 12, 24, 36, 48, 60 months

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				